Patient Name	**: Mrs. KUSALTA THAPA**	PateintID	**: LAB/0004733**
Age/Gender	: 28 Y/Female	Visit No	: SIL2511200293
Refer Client	: LDPLSL171 AAROGYAM DIAGNOSTICS	Collected On	: 20/Nov/2025 08:30 PM
Refered By	: N/A	Received On	: 21/Nov/2025 11:50 AM
Doctor Name	: Dr. N.K.KEJRIWAL.MS,MCH	Reported Date	: 21/Nov/2025 03:08 PM
SampleType	: Serum - SL104133		

SEROLOGY

Test Name	Result	Unit	Bio. Ref. Interval
DENGUE FEVER ANTIBODIES IGG & IGM- ELISA			
DENGUE FEVER ANTIBODIES IGG- ELISA ELISA	0.14	COI	<0.9 Negative 0.9-1.1 Equivocal >1.1 Positive
DENGUE FEVER ANTIBODIES IGM-ELISA ELISA	0.17	COI	<0.9 Negative 0.9-1.1 Equivocal >1.1 Positive

CLINICAL NOTES

Dengue viruses belong to the family Flaviviridae and have 4 subtypes (1-4). Dengue virus is transmitted by the mosquito Aedes aegypti and Aedes albopictus, widely distributed in Tropical and Subtropical areas of the world. Dengue is considered to be the most important arthropod borne viral disease due to the human morbidity and mortality it causes. The disease may be subclinical,
self limiting, febrile or may progress to a severe form of Dengue hemorrhagic fever or Dengue shock syndrome.

DENGUE INFECTION	ANTIBODY DETECTED POST ILLNESS	
	IgM	IgG
Primary	5 th -10th day	14th day & persists for life
Secondary	4 th -5 th day	14th day & persists for life

Note:
1. Recommended test is NS1 Antigen by ELISA in the first 5 days of fever. After 7-10 days of fever, the recommended test is Dengue fever antibodies IgG & IgM by ELISA
2. Cross reactivity is seen in the Flavi virus group between Dengue virus, Murray Valley encephalitis,Japanese encephalitis, Yellow fever & West Nile viruses

DENGUE FEVER NS1 ANTIGEN-ELISA			
DENGUE FEVER NS1 ANTIGEN-ELISA ELISA	**4.27**	COI	<0.9 Negative 0.9-1.1 Equivocal >1.1 Positive

CLINICAL NOTES:-Dengue virus (DV) is a globally distributed flavivirus with 4 distinct serotypes (DV-1, -2, -3, -4) and is primarily transmitted by the Aedes aegypti mosquito, found throughout the tropical and subtropical regions of over 100 countries.

Following dengue infection, the incubation period varies from 3 to 7 days and while some infections remain asymptomatic, the majority of individuals will develop classic dengue fever. Symptomatic patients become acutely febrile and present with severe musculoskeletal pain, headache, retro-orbital pain, and a transient macular rash, most often observed in children. Fever defervescence signals disease resolution in most individuals. However, children and young

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Test Name	Result	Unit	Bio. Ref. Interval

adults remain at increased risk for progression to dengue hemorrhagic fever and dengue shock syndrome, particularly during repeat infection with a new DV serotype.

Detection of the DV nonstructural protein 1 (NS1) has emerged as an alternative biomarker to both serologic and molecular based techniques for diagnosis of acute DV infection. NS1 antigenemia is detectable within 24 hours and up to 9 days following symptoms onset. This overlaps with the DV viremic phase and NS1 is often detectable prior to IgM seroconversion. Concurrent evaluation for the NS1 antigen alongside testing for IgM- and IgG-class antibodies to DV (DENGM) provides optimal diagnostic potential for both early and late dengue disease.

Interpretation

Positive:The presence of dengue nonstructural protein 1 (NS1) antigen is consistent with acute-phase infection with dengue virus. The NS1 antigen is typically detectable within 1 to 2 days following infection and up to 9 days following symptom onset. NS1 antigen may also be detectable during secondary dengue virus infection, but for a shorter duration of time (1-4 days following symptom onset).

Negative:The absence of dengue NS1 antigen is consistent with the lack of acute-phase infection.The NS1 antigen may be negative if specimen is collected immediately following dengue virus infection

Cautions: Results should be used in conjunction with clinical presentation and exposure history. Though uncommon, false-positive nonstructural protein 1 (NS1) results may occur in individuals with active infection due to other flaviviruses, including West Nile virus and yellow fever virus.Negative NS1 antigen results may occur if the specimen was collected >7 days following symptom onset. Serologic testing for the presence of IgM and IgG antibodies to DV is recommended in such cases.

Note:

1. Recommended test is NS1 Antigen by ELISA in the first 5 days of fever. After 7-10 days of fever, the recommended test is Dengue fever antibodies IgG & IgM by ELISA
2. Cross reactivity is seen in the Flavi virus group between Dengue virus, Murray Valley encephalitis, Japanese encephalitis, Yellow fever & West Nile viruses

*****End of Report*****

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